

Parenthood Timing and Gender Inequality

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Motivation

Gender inequality in OECD labor markets typically emerges with parenthood

- Understanding the role of parenthood is crucial for addressing gender inequity

Quantifying the effects of parenthood is challenging due to selection:

- Parents may differ systematically from those who are and/or remain childless

Addressing selection requires quasi-experimental fertility variation:

- A setting where pregnancy is as-good-as-random (various quasi-experiments)
- Compare those who conceive at that moment to those who do not

But many women who do not conceive initially eventually become mothers

- Existing evidence identifies a weighted average of parenthood and timing effects

These forces may work in opposite directions and require distinct policies

This Paper

What are the effects of parenthood and its timing?

- How would outcomes differ if individuals remained childless or delayed childbirth?
- 1. New approach to estimate treatment effects in sequential quasi-experiments
 - Settings where previously unassigned individuals can repeatedly undergo assignment
 - E.g. a series of conception attempts
- 2. Empirical evidence using novel Dutch administrative data
 - Focus on couples undergoing intrauterine insemination
 - Common and minimally invasive infertility treatment
- 3. Quantify and compare the effects of parenthood and its timing
 - How does parenthood shape women's outcomes and gender inequality?
 - Does later childbearing mitigate or amplify losses?

Literature and Contribution

1. Existing quasi-experimental studies average parenthood and timing effects

- Hotz et al. (2005); Agüero & Marks (2008); Cristia (2008); Miller (2011); Lundborg et al. (2017); Bensnes et al. (2023); Gallen et al. (2023); Lundborg et al. (2024)

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3. Separating treatment and timing effects in quasi-experiments is a common challenge

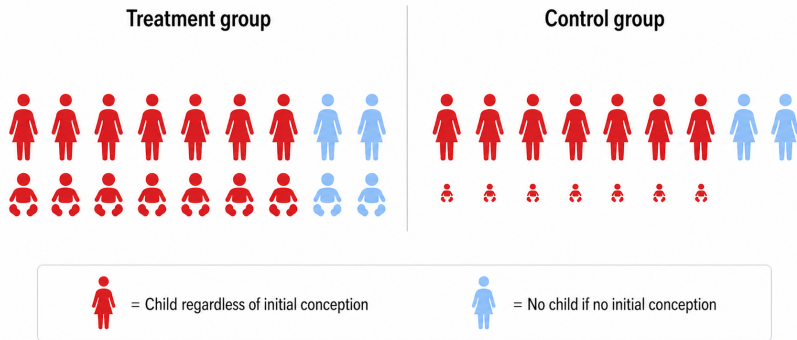
- Education programs with multiple admission cycles, assignment to judges, promotion tournaments

Method applicable to many other settings with sequential quasi-experiments

Typical Quasi-Experiment

At a given moment, some women conceive (treatment), others do not (control).

- Control group: remain childless or conceive later



Mothers differ in age at first birth, career stage at birth, and potentially # of children.

- Comparison mixes motherhood vs. childlessness with earlier vs. later motherhood

Baseline Reduced-Form Setup

Address selection using a quasi-experiment: women undergoing artificial insemination.

- Outcome of the first procedure $Z_1 \in \{0, 1\}$
- Parenthood indicator $D \in \{0, 1\}$
- Potential labor market outcomes $Y_{z_1}(d)$
- Effect of parenthood: $Y_1(1) - Y_0(0)$
- Effect of parenthood timing: $Y_1(1) - Y_0(1)$

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Standard reduced form argument:

1. Assume success of the first procedure is random (conditional on age)
2. Compare women whose first procedure succeeds to those whose fails:

$$\mathbb{E}[\text{Parenthood effect} \mid D = 0 \text{ if } Z_1 = 0]c + \mathbb{E}[\text{Timing effect} \mid D = 1 \text{ if } Z_1 = 0](1 - c)$$

- $c = \Pr(\text{no child if } Z_1 = 0) \approx 0.25$ in practice

Method Overview

Key observation: many women conceive through subsequent IUIs.

- Variation in later IUI success could help separate treatment and timing effects
- Leveraging this variation is challenging because the number of IUIs is endogenous

Result 1: Selection into subsequent IUIs can be addressed using the observed IUI #

- Identify average outcomes in the scenario where entire IUI sequence fails
- Many women remain childless; some conceive (much later) via natural means

Result 2: Worst-case bounds allow to separate treatment and timing effects

- Bound the effect of conceiving at the first IUI vs. remaining childless
- Bound the effect of conceiving at the first IUI vs. after the IUI sequence

No assumptions required on selection into subsequent IUIs and natural conception or on the structure of treatment/timing effects within or across individuals.

Statistical Model

Women differ in two unobserved characteristics:

- “Willingness” to undergo IUIs, $W \in \{1, \dots, \overline{w}\}$
 - Would undergo W IUIs for the first child if all IUIs failed
- “Reliance” on IUIs, $R \in \{0, 1\}$
 - No child if all IUIs fail, $R = 1$

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Observables:

- IUI j success indicator, Z_j , for procedures before having any children
- Number of realized IUIs:

$$A = \min(\{j : Z_j = 1\} \cup \{W\})$$

- Parenthood indicator:

$$D = \max(Z_A, 1 - R)$$

Sequential Unfoundedness

Assumption (Sequential Unfoundedness)

$$(Y_1(1), Y_0(0), R, W) \perp\!\!\!\perp Z_j | A \geq j.$$

In words: once sperm at IUI j are implanted, whether this results in a conception is as-good-as-random

- **The decision to undergo the procedure can be endogenous**
- $Y_1(1), Y_0(0), R$ and W can be related
- Main method relaxes to covariate-conditional version: age at procedure, technology

Simple World: Max 2 IUs, All Reliers

$$W = 1$$

(willing to try once)

$$Z_1 = 1$$

$$Z_1 = 0$$

Simple World: Max 2 IUs, All Reliers

$$W = 1$$

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$$Z_1 = 1$$

$$Z_1 = 0$$

$$W = 2$$

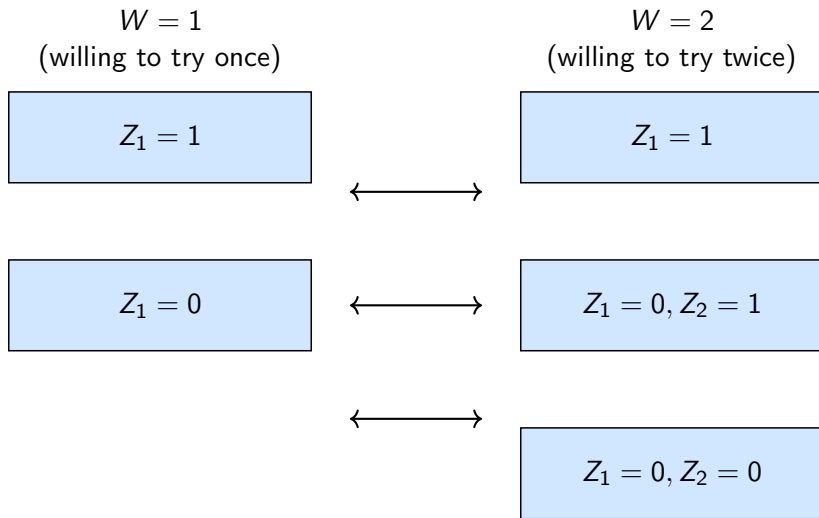
(willing to try twice)

$$Z_1 = 1$$

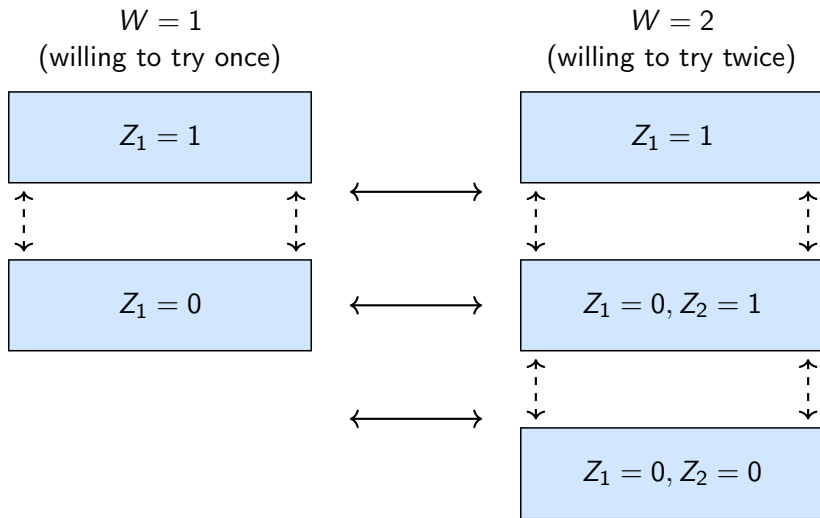
$$Z_1 = 0, Z_2 = 1$$

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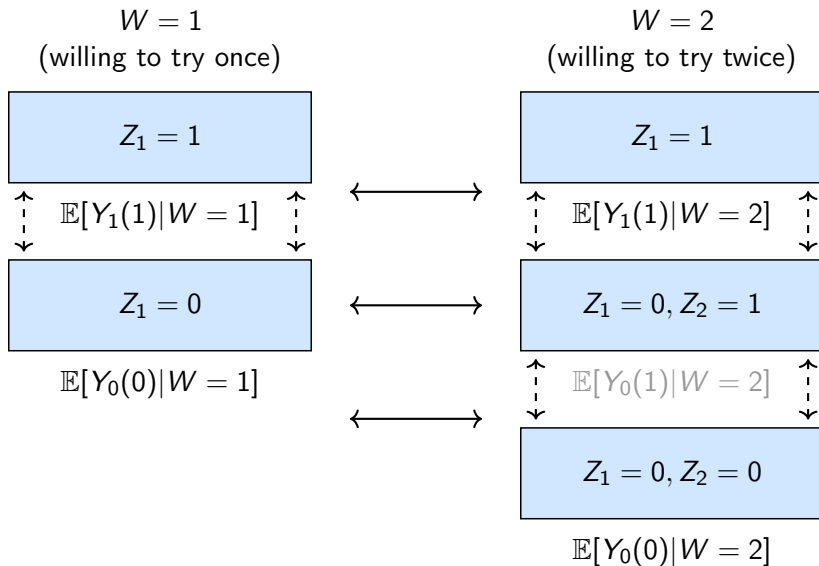
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Simple World (Observed): Max 2 IUs, All Reliers

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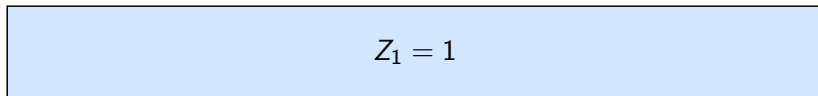
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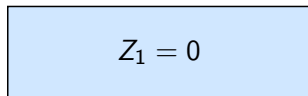
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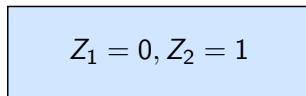
$W = 2$
(willing to try twice)



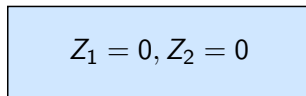
$\mathbb{E}[Y_1(1)]$



$\mathbb{E}[Y_0(0)|W = 1]$

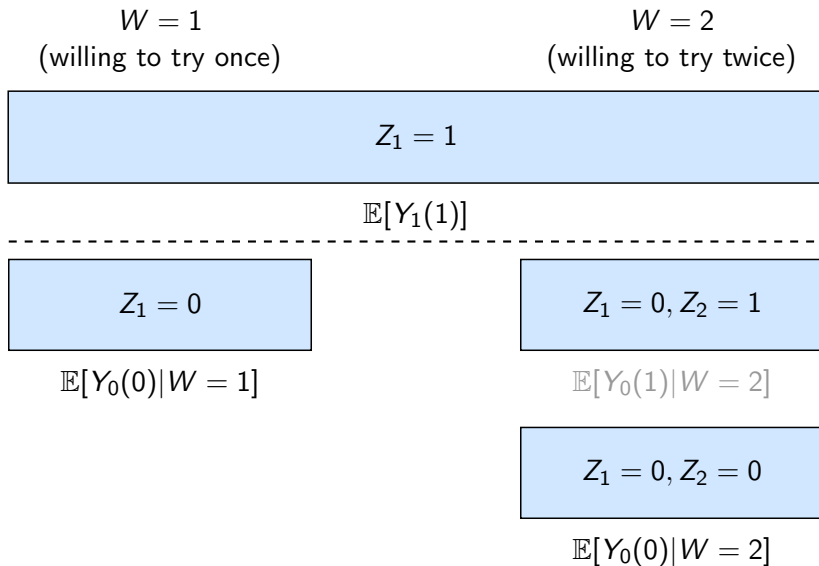


$\mathbb{E}[Y_0(1)|W = 2]$



$\mathbb{E}[Y_0(0)|W = 2]$

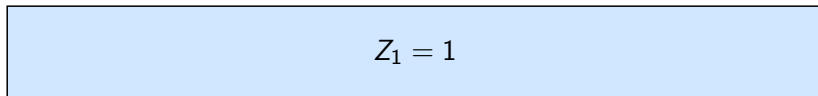
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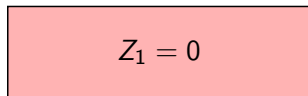
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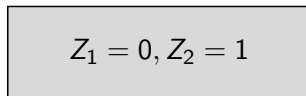
$W = 2$
(willing to try twice)



$\mathbb{E}[Y_1(1)]$

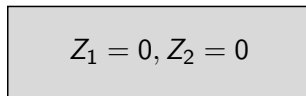


$\mathbb{E}[Y_0(0)|W = 1]$



$\mathbb{E}[Y_0(1)|W = 2]$

$Pr(W = 1) =$



$\mathbb{E}[Y_0(0)|W = 2]$

Simple World: Max 1 IUI with Non-reliers

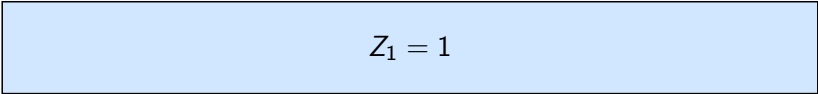
$R = 1$
(no child if fail)

$R = 0$
(child if fail)

Simple World: Max 1 IUI with Non-reliers

$R = 1$
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$$Z_1 = 1$$

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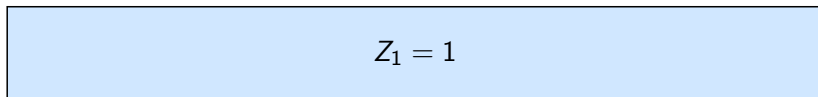
$$Z_1 = 0, D = 0$$

$$Z_1 = 0, D = 1$$

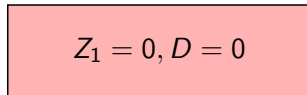
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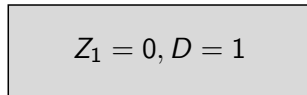
$R = 0$
(child if fail)



Distribution of $Y_1(1)$



$\mathbb{E}[Y_0(0)|R = 1]$

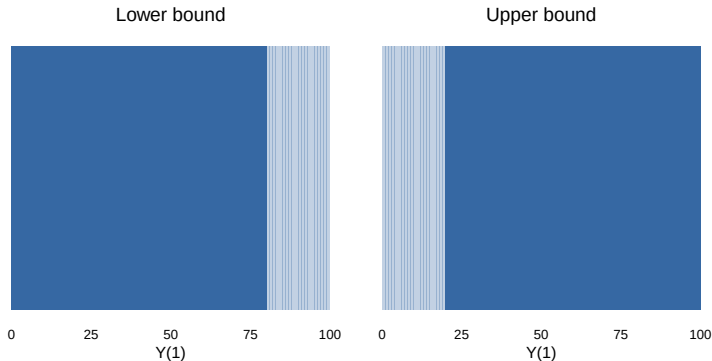


$\mathbb{E}[Y_0(1)|R = 0]$

$$Pr(R = 1) = \frac{\text{red square}}{\text{red square} + \text{gray square}}$$

Intuition: Motherhood Outcome $Y_1(1)$

1. Treated group is a representative sample but their types are unobserved
2. Identify $\Pr(R = 1) = 0.8$ on control group
3. Assume most extreme distributions of types in treated group
4. Bound $\mathbb{E}[Y_1(1)|R = 1]$



Argument for timing effects is symmetric.

Background and Data

Assisted conception procedures →

- In-vitro fertilization: invasive medical procedure, first 3 free
- Intrauterine insemination: direct sperm injection, minimally invasive, free

Dutch family policies and labor market similar to OECD average →

- 16 weeks maternity + pregnancy leave, 1 week paternity leave

Data combining ACP medical records with tax records →

- Work hours and income include leave; results for hours corrected for uncertainty
- 15,523 cohabiting opposite-sex couples
- Balance: ACP success at each attempt uncorr. with past outcomes cond. on age
- Reliers make up $\sim 50\%$ of the sample

Balance in 1st IUI first

Balance in later IUIs

Success probabilities per attempt

Effect of Parenthood: Women

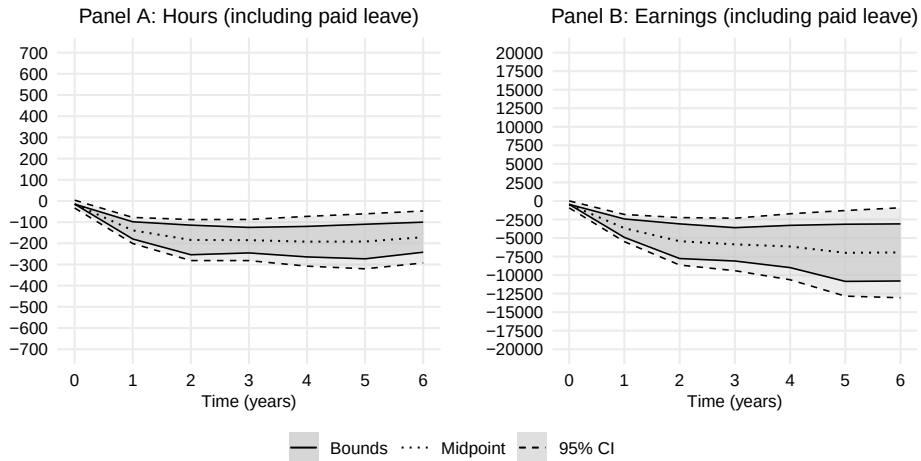


Figure 1: Parenthood Effect Bounds for Women

Effect of Parenthood: Men



Figure 2: Parenthood Effect Bounds for Men

Share of Gender Inequality Caused by Parenthood



Figure 3: Effect on Gender Gap relative to Gap Under Parenthood

Timing Effects

So far:

- Compare earlier-treated vs. control outcomes

Next:

- Compare non-relied earlier-treated vs. later-treated outcomes

On average, women whose IUIs fail conceive 3.1 years later.

I focus on cumulative effects from year 3 onward:

- Interpretation: total effect of delay up to a given point in the lifecycle

Two components:

- (a) At year 3: cumulative effect of no child vs. a child conceived 3 years ago
- (b) At year $3 + t$: cumulative effect of motherhood starting $3 + t$ vs. t years ago

Difference (b) – (a): after motherhood, do women who become mothers later do better or worse than those who become mothers earlier?

Cumulative Effect of Parenthood Timing: Women

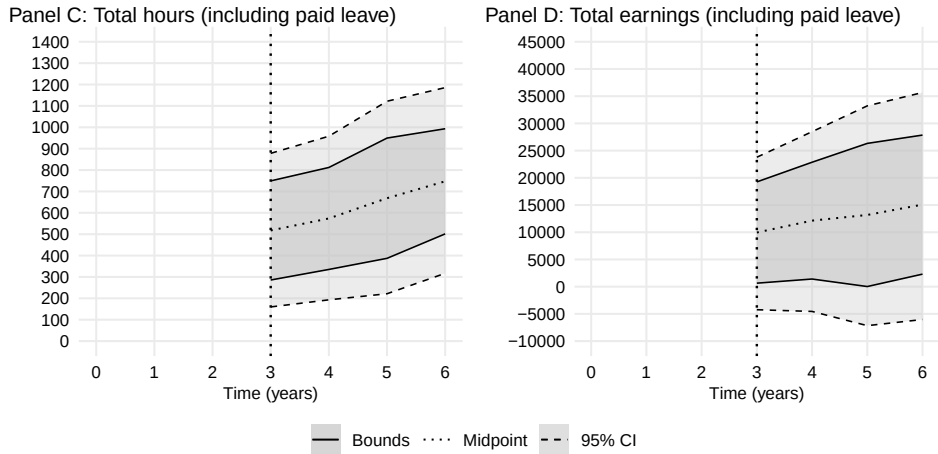


Figure 4: Parenthood Timing Effect Bounds for Women

How Much Does Delayed Childbearing Mitigate Career Costs?

Focus on midpoints of bounds

- Necessarily suggestive; comparing short run estimates for reliers and non-reliers

Before delayed birth:

- Absence of children: +16% annual hours, +19% earnings

After delayed birth:

- Gains shrink to +5% hours, +2% earnings

Lifetime implications (33-year horizon from age 32)

- 1-year delay $\Rightarrow$ +5.5% hours, +2.5% earnings
- 5-year delay $\Rightarrow$ +7% hours, +4.5% earnings

Baseline long-run parenthood losses: -15% hours, -20% earnings

Five-year delay reduces losses to: -9% hours, -16% earnings

Conclusion

Results:

- Motherhood accounts for up to half of gender inequality in hours and income
- Delayed childbearing mitigates women's losses

External relevance:

- IUI is common; the sample matches the population on observables
- Many women would undergo IUI if natural conception failed
- Parenthood effects exceed Scandinavian IVF/contraceptive-failure estimates

Policy:

- Enabling later childbearing may mitigate gender inequality
- Some gaps might persist even in the absence of children

Extensions

- **Selection into parenthood** [Formal procedure](#) [Estimates](#)
- **Relation to partial identification literature** [Comparison](#)
- **Robustness** [Childless final period](#) [De-aging partners](#) [Mental health](#) [Bounds for non-depressed](#)

How Existing Work Isolates Parenthood Effects

Parenthood effects may depend on current lifecycle stage and duration of motherhood:

$$\tau(\text{age, duration})$$

- **Age** = current career stage
- **Duration** = first child age
- **Age x Duration** = career stage at first birth

A) Standard IV approach (Lundborg et al., 2017, 2024):

$$\tau(\text{age, duration}) = \tau(\text{age})$$

B) Recursive IV approach (Bensnes et al., 2023; Gallen et al., 2023):

$$\tau(\text{age, duration}) = \tau(\text{duration}) \quad (\text{and homogeneous effects})$$

- A) requires that **duration** does not matter, B) that **career stage** does not matter
- Both A) and B) require that **career stage at parenthood** does not matter
- Estimates differ substantially across approaches (Bensnes et al., 2023)

Bounding τ_{ATR}

Construct the moment:

$$m^L(G, \eta^0) = Y 1_{\{Y < q(r(X_1), X_1)\}} \frac{Z_1}{e_1(X_1)} - Y(1 - D) \prod_{j=1}^A \frac{(1 - Z_j)}{(1 - e_j(X_j))}$$

- G is the observed data vector
- η^0 contains the following:
 - $e_j(X_j) = \Pr(Z_j = 1 | X_j)$
 - $q(r(X_1), X_1)$ is the $r(X_1)$ -th quantile of Y given X_1 and $Z_1 = 1$
 - $r(X_1)$ identifies the covariate-conditional relier share

Assumption (Conditional Sequential Unconfoundedness)

$(Y(k), R, W) \perp\!\!\!\perp Z_j \mid X_j$ for all j, k , and $X_j, A \geq j$.

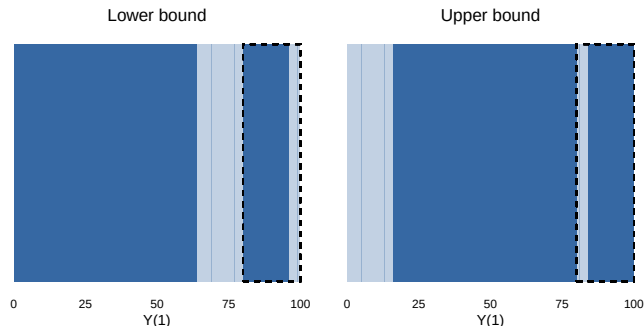
Theorem (Lower Bound)

Under conditional sequential unconfoundedness and regularity, the sharp lower bound on τ_{ATR} is $\mathbb{E}[m^L(G, \eta^0)] / \mathbb{E}[r(X_1)]$.

Intuition: Motherhood Outcome $Y_1(1)$ —Covariates

Pre-IUI covariates can help narrow the bounds:

- Can identify relier share at each covariate value
- Baseline bounds assume extreme scenarios where reliers have highest or lowest treated outcomes
- These distributions of treated outcomes might be inconsistent with conditional relier shares



Estimating the Bounds

Distribution of $m^L(G, \eta^0)$ is complicated by $q(r(X_1), X_1)$

- Semenova (2023) addresses a closely related inference challenge
- Double/debiased machine learning approach
 1. Adjust $m^L(G, \eta^0)$ to make it insensitive to small error in $q(r(X_1), X_1)$
 2. Sample splitting
- Asymptotic inference as if $q(r(X_1), X_1)$ was known

New moment:

$$\psi^L(G, \xi^0) = m^L(G, \eta^0) + \text{corr}(G, \xi^0)$$

Identifies same parameter:

$$\mathbb{E}[\psi^L(G, \xi^0)] = \mathbb{E}[m^L(G, \eta^0)]$$

Insensitive to estimation error in $q(r(X_1), X_1)$:

$$\partial_{q(\cdot)} \mathbb{E}[\psi^{L+}(G, \xi_r) | X_1] |_{\xi_r = \xi_r^0} = 0 \text{ a.s.}$$

Assisted Conception Procedures

- IUI (main procedure): sperm injected into uterus
 - Minimally invasive, primary ACP in most countries
 - “Free” in NL
- IVF (secondary procedure): embryo inserted into uterus
 - Invasive treatment, performed under sedation/anesthesia
 - Eggs retrieved through the vaginal wall using a specialized needle
 - In NL, first 3 free; each subsequent costs between 1000 and 4000 EUR

[Back](#)

Institutions

- Dutch family friendly policies similar to OECD average
 - 16 weeks of fully paid pregnancy+maternity leave
 - 1 week of paternity leave
 - Average time in child care similar to OECD average
 - Net child care cost 10% median household income
- Dutch employment intensity similar to OECD average
 - Employment among parents and non-parents relatively high
 - Part time work much more common
 - Approximately 15% two-parent families have both partners working part-time

[Back](#)

Data

Administrative data from Statistics Netherlands

- Comprehensive hospital records cover fertility treatments from 2012 to 2017: procedure date and type
 - Success imputed as having child born within 10 months
- Tax records cover work hours and income from 2011 to 2023
 - Include maternity leave and pay
 - Main bounds account for uncertainty around actual work hours
- Birth dates, legal family connections, cohabitation
- Dispensed medication registry

Main sample: cohabiting opposite-sex couples undergoing IUI for their first child between 2013 and 2016: 15,523

[Back](#)

Table 1: First IUI Outcomes and Descriptives

	Success (1)	Fail (2)	Dif. (1)-(2)	IPW dif. (1)-(2) cond.	Rep. (5)	Suc. vs rep. (1)-(5)
Work (W)	0.912 [0.283]	0.916 [0.277]	-0.004 (0.008)	-0.009 (0.008)	0.936 [0.244]	-0.024 (0.007)
Work (P)	0.894 [0.307]	0.885 [0.319]	0.009 (0.009)	0.002 (0.009)	0.897 [0.304]	-0.002 (0.008)
Hours (W)	1300.012 [547.832]	1298.876 [558.316]	1.136 (15.730)	-1.951 (16.119)	1310.923 [544.468]	-10.911 (14.554)
Hours (P)	1513.337 [635.121]	1494.541 [656.050]	18.796 (18.457)	3.345 (19.041)	1497.603 [651.043]	15.734 (17.403)
Earn. 1000s EUR (W)	29.358 [18.000]	29.648 [18.911]	-0.290 (0.531)	0.203 (0.561)	26.555 [15.989]	2.803 (0.427)
Earn. 1000s EUR (P)	38.082 [25.425]	38.060 [26.525]	0.022 (0.745)	0.322 (0.774)	33.862 [24.148]	4.220 (0.646)
Bachelor deg. (W)	0.512 [0.500]	0.494 [0.500]	0.018 (0.014)		0.518 [0.500]	-0.007 (0.013)
Bachelor deg. (P)	0.425 [0.494]	0.410 [0.492]	0.014 (0.014)		0.430 [0.495]	-0.005 (0.013)
Age (W)	31.373 [3.889]	32.060 [4.265]	-0.687 (0.119)		28.840 [3.896]	2.533 (0.104)
Age (P)	34.088 [4.968]	34.856 [5.500]	-0.768 (0.154)		31.415 [4.803]	2.673 (0.128)
Observations	1,411	11,323			171,180	
Joint <i>p</i> -val.			0.001	0.955		0.000

Note: *Success* – average among women whose first IUI succeeded; *Fail* – average among women whose first IUI failed; *Dif.* – difference between *Success* and *Fail*; *IPW dif.* – difference adjusted for age and education using inverse probability weights from the baseline specification; *Rep.* – average in representative sample of women who conceived their first child without assisted conception procedures; *Suc. vs rep.* – difference between *Success* and *Rep.*. Reference year: year of first IUI (IUI sample); 9 months before first birth (representative sample). IUI sample: women who underwent intrauterine insemination for their first child between 2013 and 2016, with no prior assisted conception procedures, cohabiting with a male partner in the year prior to the reference year. Representative sample: women with no assisted conception procedures before first birth, cohabiting with a male partner in the year prior to the reference year, with reference year between 2013 and 2016. Labor market outcomes measured in the year before the reference year; age measured in the reference year. *Bachelor deg.* – indicator for completing a bachelor's degree. *Earn.* – earnings, (W) – woman, (P) – partner. Standard deviations in brackets. Standard errors in parentheses.

Balance in Subsequent ACPs

Table 2: Balance in Later ACPs

	Z ₂	Z ₃	Z ₄	Z ₅	Z ₆	Z ₇	Z ₈	Z ₉	Z ₁₀
Work (W)	0.009 (0.010)	-0.004 (0.011)	0.022 (0.011)	0.014 (0.012)	0.039 (0.012)	-0.003 (0.017)	-0.011 (0.018)	0.022 (0.019)	0.030 (0.024)
Work (P)	0.006 (0.010)	0.016 (0.010)	0.012 (0.012)	0.020 (0.012)	-0.004 (0.015)	-0.004 (0.015)	-0.019 (0.019)	0.017 (0.020)	0.030 (0.027)
Hours (W)	32.885 (18.721)	-4.482 (20.032)	52.999 (21.045)	41.332 (22.686)	81.957 (25.131)	11.894 (31.187)	-18.836 (32.937)	72.659 (38.210)	24.819 (48.490)
Hours (P)	21.655 (21.018)	24.730 (21.089)	23.756 (23.574)	38.965 (25.255)	9.666 (30.585)	-6.580 (31.513)	-28.458 (37.976)	30.525 (44.856)	43.722 (52.821)
Income 1000s € (W)	1.481 (0.615)	-0.015 (0.624)	1.685 (0.767)	1.802 (0.830)	2.086 (0.913)	0.150 (1.000)	-0.043 (1.092)	0.866 (1.234)	-0.444 (1.629)
Income 1000s € (P)	-0.749 (0.835)	1.002 (0.912)	2.040 (1.066)	0.800 (1.115)	0.774 (1.424)	0.025 (1.424)	0.259 (1.563)	-0.324 (1.737)	0.149 (2.203)
Observations	12,974	10,774	8,726	6,977	5,411	3,944	2,723	1,850	1,174
Joint <i>p</i> -val.	0.175	0.976	0.234	0.303	0.140	1.000	0.956	0.704	0.917

Note: Each column describes the difference in average characteristics between women for whom the respective ACP succeeds and those for whom it fails, among those who undergo the procedure, using inverse probability weights for each ACP following the main specification. Labor market outcomes and age measured year before first treatment. (W) - woman, (P) - partner. Standard errors in parentheses.

Estimated Success Probabilities

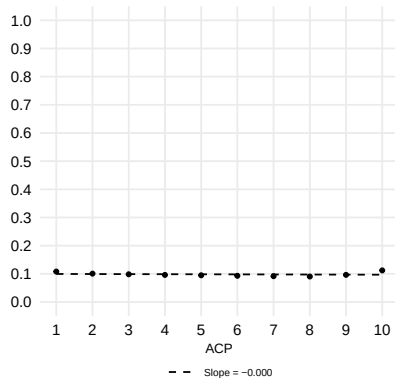


Figure 5: Estimated Success Probabilities

Comparison with Lee (2009)

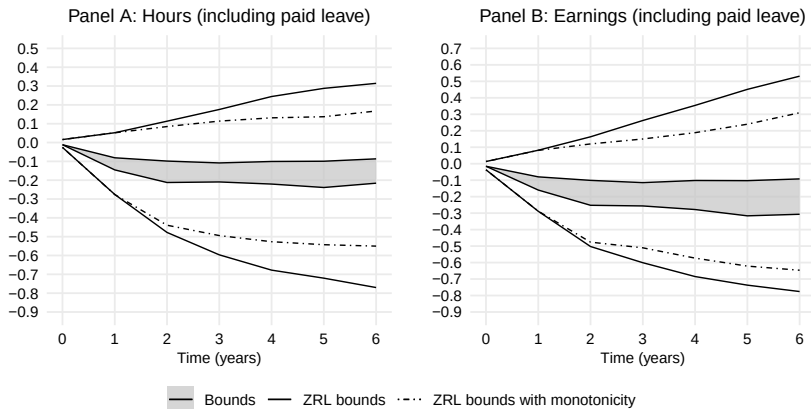


Figure 6: Comparison with Lee (2009) Bounds for Effects on Women

Quantifying Selection Using the Event Study Approach

Kleven et al. (2024) compare mothers t years after childbirth to women one year before

- Most gender inequality is explained by these differences between women
 - Event study estimates may be biased due to selective timing
 - But differences from IV/bounds need not imply bias, even in the same sample
1. I proxy fertility timing using the timing of first IUI
 2. Compare the actual trajectories of women who attempted conception but remained childless with those imputed from women who are about to attempt conception but ultimately remain childless
- **Event study comparison of women with different timing absent children**
 - **Same population as bounds: selection vs causal effects directly comparable**

[Back \(extensions\)](#)

[Conclusion](#)

Placebo Event Study

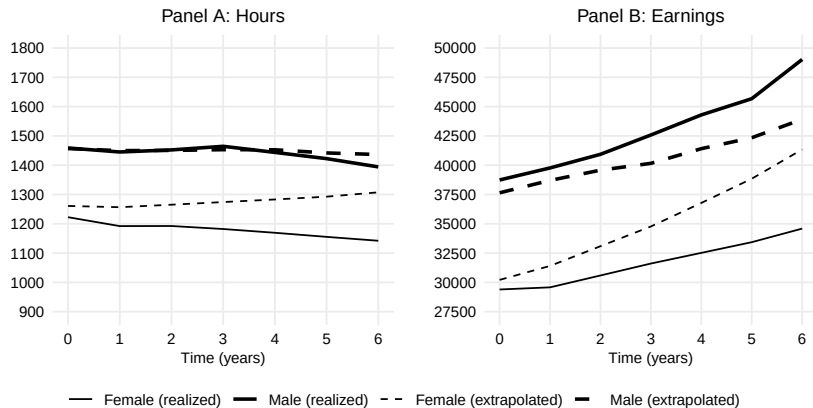


Figure 7: Placebo Event Study

Gender Inequality: Causal vs Selection

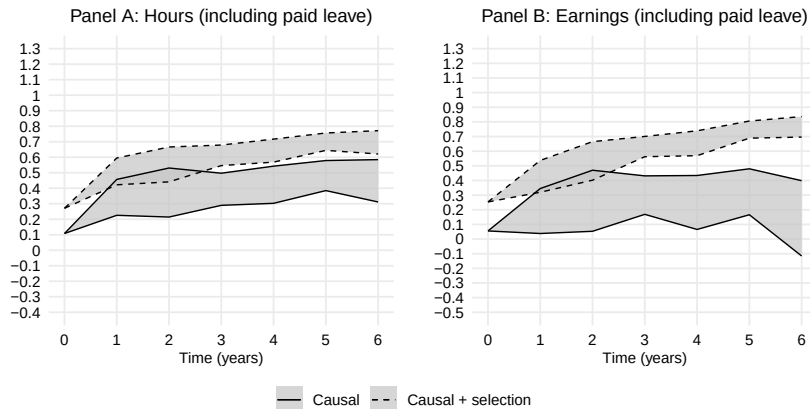


Figure 8: Share of Gender Inequality Explained by Selection and Parenthood

Mental Health and Assisted Conception

Mental health consequences associated with failure to conceive are a part of the story:

- Unmet fertility goals may negatively impact mental health, and in turn, labor market outcomes

There are, however, additional concerns:

- Mental health issues caused specifically by failed conception or ACPs (external)
 - Focusing on artificial insemination helps mitigate this
- Large impacts unique to ACP families (external)
- Worsened mental health by threatening monotonicity (internal)

In practice, these impacts are likely small (Lundborg et al., 2024)

[Antidepressant uptake](#)

[Back \(extensions\)](#)

[Conclusion](#)

Monotone Bounds for Non-depressed Childless Women



Figure 9: Monotone Bounds for Women Who Would Not Uptake Antidepressants if They Were to Remain Childless

Effect of IUI Failure on Antidepressant Use

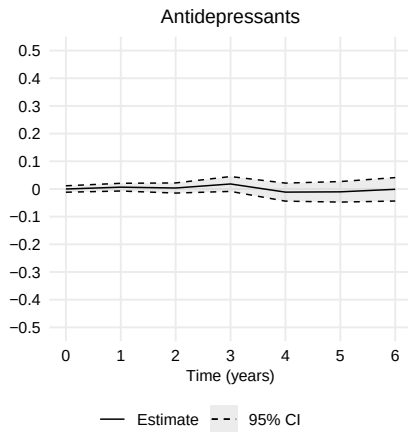


Figure 10: Estimates for effect on antidepressant take-up

Monotone Bounds: Women Who Remain Childless

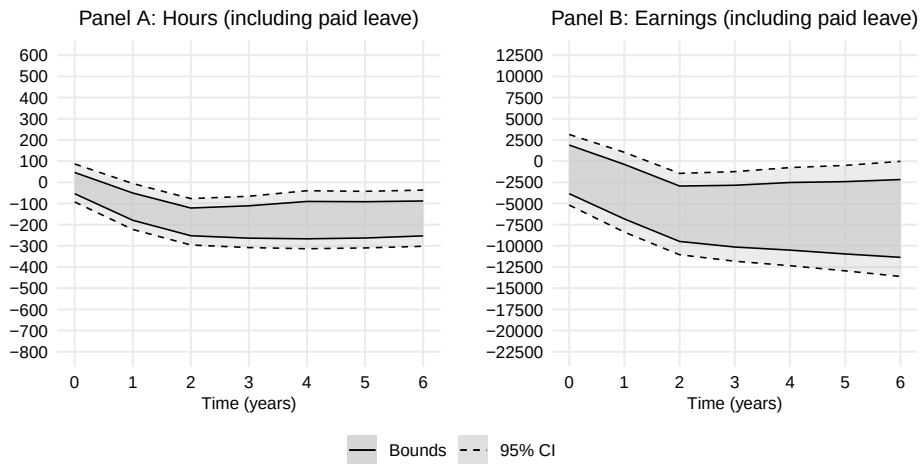


Figure 11: Monotone Bounds Using Completed Fertility

Parenthood Effect Bounds: Correcting for Partner's age

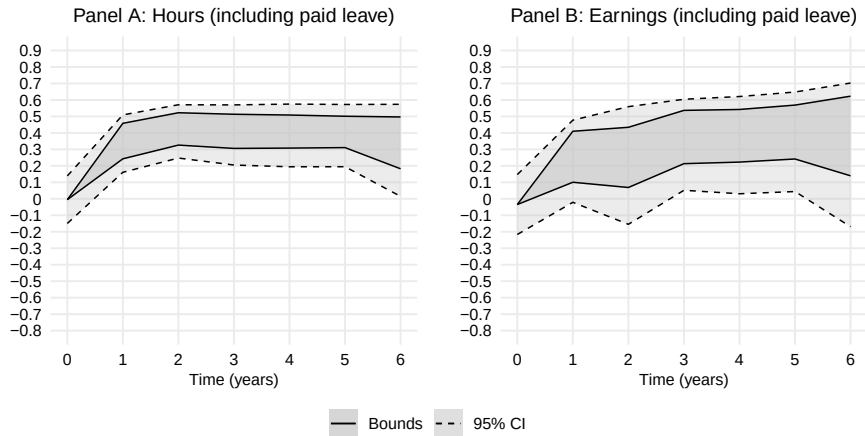


Figure 12: Bounds Measuring Male Income at Same Age as Female

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